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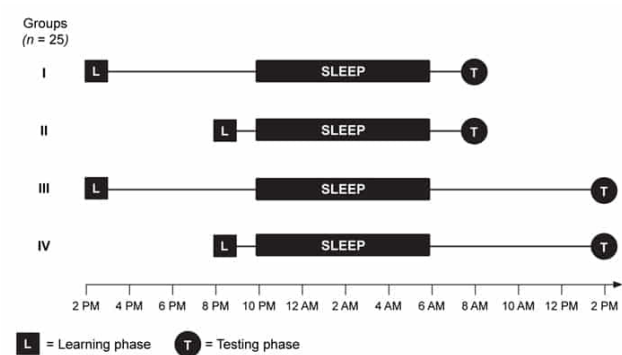


Figure 1 Experimental design for each of the 4 groups

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words "plucky" (meaning brave) and "tocsin" (meaning an alarm) to an image of a toy fire truck, making it easier to recall the toy object and both associated vocabulary definitions.

Table 1 Average Recall Accuracy, per Group

Groups	Average object recall accuracy (%)	Average vocabulary recall accuracy (%)
I	52	42
II	69	60
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IV	64	52

The recall of which of the following should be most subject to serial position effects?

- ☐ A. The vocabulary words presented at the beginning and the end of the list
- ☐ B. The toy objects presented at the beginning and the end of the sequence
- ☐ C. The vocabulary words that were the most familiar to the subjects
- ☐ D. The toy objects to which the subjects had a personal connection

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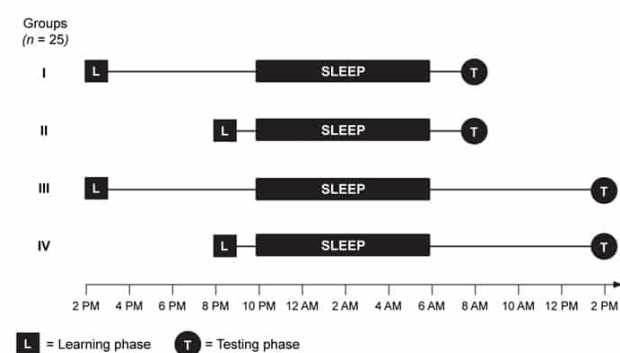


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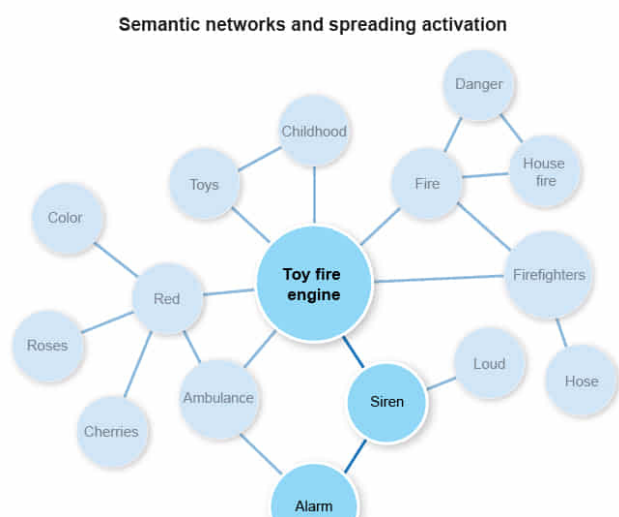
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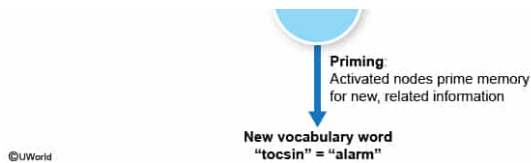
If the students who related the toy objects and definitions to each other had higher-than-average recall scores, to which of the following might this be attributed?

- I. Spreading activation
 - II. Semantic networks
 - III. Source monitoring
- ☐ A. I only [4%]
☐ B. II only [14%]
☐ C. III only [1%]
☒ D. I and II only [78%]

Correct
78% answered correctly

Explanation:





Semantic long-term memory appears to be organized as a **network** of **interconnected nodes** containing factual concepts (eg, colors, objects). The organization and relationship between nodes (how linked or connected they are in memory) is *unique to each individual*. For example, an individual with an uncle who is a firefighter may think of "uncle" when viewing a fire engine, which would not occur for most people.

(Number I) The **spreading activation model** suggests that when a node in the semantic network is activated (eg, viewing a picture of a toy fire engine), nodes directly connected to that node (eg, firefighter, alarm) are activated as well, which is known as **priming**. Spreading activation could account for better recall when students related the toy objects and definitions to each other.

(Number II) Each individual has a uniquely organized **semantic network** according to the personal meaning associated with each node. It tends to be *easier* to recall information that is semantically relevant (ie, meaningful). Therefore, better recall for students who related the toy objects and definitions to each other may be attributable to the use of semantic networks.

(Number III) Source monitoring refers to errors that occur when an individual incorrectly attributes a memory to the wrong source. For example, an individual who reads on the AAMC website (source) that the MCAT is adding a new behavioral sciences section, but when recalling this information remembers hearing it from a friend (incorrect source), has committed a source monitoring error. Source monitoring errors *do not* explain why those students who related the toy objects and definitions to each other had higher-than-average recall scores. Their strategy and improved recall can best be explained by spreading activation, semantic networks, and priming.

Educational objective:

Semantic long-term memory is thought to be stored as a network of interconnected concepts (nodes) in a semantic (meaning-based) network. Spreading activation suggests that when a node in the semantic network is activated, the nodes directly connected to that node are then activated as well (ie, priming).

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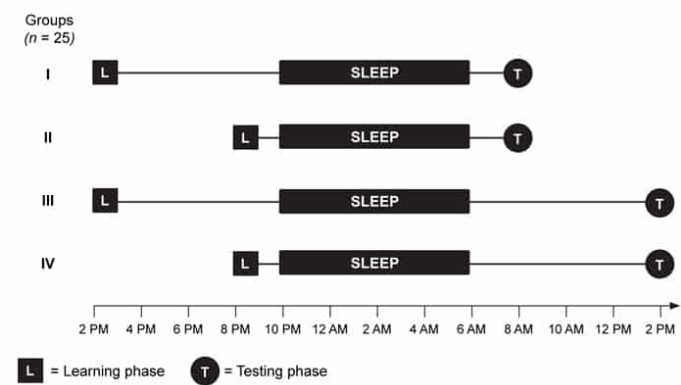


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Which of the following, had it occurred, would be the best example of the social desirability bias in this study?

- ☐ A. Researchers subconsciously allowed Group II to view the toys slightly longer than the other groups. [8%]
- ☐ B. Researchers selected only college students with regular 10 pm–6 am sleep schedules to participate. [14%]
- ☐ C. Some participants reported using their typical strategy for vocabulary memorization in this task. [2%]
- ☒ D. Some participants did not disclose that they stayed out past 10 pm the night of the experiment. [74%]

Correct
74% answered correctly

Explanation:

Examples of bias in scientific research	
Bias on the part of the study	Bias on the part of the subjects
• Experimenter bias	• Response bias

- Sampling bias
- Question-order bias

- Social desirability bias
- Hawthorne effect

Bias in scientific research occurs when something that is not accounted for or held constant influences the data. Although many types exist, bias generally falls into 2 categories: bias on the part of the study (eg, experimenter bias, sampling bias) or bias on the part of the subjects (eg, social desirability bias, response bias).

Social desirability bias describes the **tendency** of **research participants** to provide the most favorable or acceptable responses to research questions. In other words, participants may **overemphasize positive behaviors** (eg, studying) while downplaying or **underreporting undesirable behaviors** (eg, consuming alcohol).

Participants who had stayed out past the time they reported going to sleep would be responding so as to seem like ideal participants, which would be an example of possible social desirability bias in this study.

(Choice A) Experimenter bias occurs when researchers deliberately or inadvertently influence the results. Often, this bias leads to results that confirm what the researcher was expecting. If the researchers expected Group II to perform the best and subconsciously gave them more time to view the toys (likely resulting in better recall), this would be an example of *experimenter bias*, not social desirability bias.

(Choice B) Sampling bias occurs when study participants are recruited from a population nonrandomly, often resulting in a nonrepresentative sample. Students with a regular 10 pm–6 am sleep schedule are not likely representative of most college students. Including only these students limits the ability to extrapolate findings to the larger college population, and is an example of *sampling bias*, not social desirability bias.

(Choice C) Mental set describes the inclination to use old methods to solve new problems, even when those methods are not appropriate for the current problem. Participants reporting that they used their typical strategy for vocabulary memorization is an example of *mental set*, not social desirability bias.

Educational objective:

Social desirability bias describes the tendency of research participants to provide the most favorable or socially acceptable responses to research questions. Participants may overemphasize positive behaviors while downplaying or underreporting undesirable behaviors.

Time spent:0 sec

Subject:
Behavioral Sciences

QID:400173

System:
Learning, Memory, and Cognition

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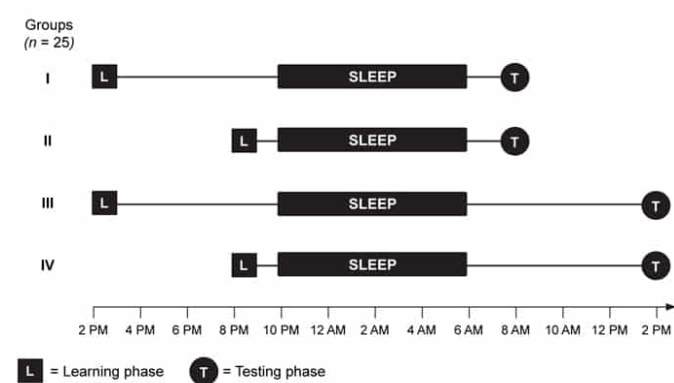


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Groups	Average object recall accuracy (%)	Average vocabulary recall accuracy (%)
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Which conclusion is LEAST supported by the data in Table 1?

- ☐ A. A shorter time interval between learning and testing results in a greater amount of recall. [4%]
- ☐ B. When sleep occurs shortly after learning, memory consolidation appears to be enhanced. [5%]
- ☐ C. Recalling vocabulary definitions was more difficult than recalling toy objects for all groups. [6%]
- ☒ D. The interval between sleep and testing matters less to recall than the method of memorization. [82%]

Correct
83% answered correctly

Explanation:

Groups	Learning-sleeping time interval (hrs)	Learning-testing time interval (hrs)	Sleeping-testing time interval (hrs)	Average object recall accuracy (%)	Average vocab. recall accuracy (%)
I	8	18	2	52	42
II	2	12	2	69	60

III	8	24	8	44	35
IV	2	18	8	64	52

In this experiment, the learning-sleeping, learning-testing, and sleeping-testing time intervals were all **independent variables**, which were **manipulated by the researcher** to see if they had **an effect on the dependent variable**, recall.

Although many studies have suggested that the method of memorization employed *is* important for memory consolidation and recall accuracy, this study did not directly test the effects of various memorization methods on memory consolidation or recall accuracy. Therefore, the conclusion that the interval between sleep and testing is less critical for memory consolidation than the method of memorization employed is *not supported* by the data in Table 1.

(Choice A) The data in Table 1 do support the conclusion that a shorter time interval between learning and testing results in better recall, because Group II, which had the shortest learning-testing time interval (12 hours), also demonstrated the best average recall accuracy for both objects (69%) and vocabulary (60%).

(Choice B) The data in Table 1 do support the conclusion that when sleep occurs shortly after learning, memory consolidation appears to be enhanced, because Groups II and IV had the shortest learning-sleeping time interval (2 hours) and also demonstrated higher average recall than Groups I and III, which had a longer learning-sleep time interval (8 hours).

(Choice C) The data in Table 1 do support the conclusion that recalling vocabulary definitions was more difficult than recalling toy objects for all groups, because for all groups the average percentage recalled was higher for objects than for vocabulary definitions.

Educational objective:

In a research study, independent variables are manipulated or controlled by the researcher and dependent variables are the outcomes that are measured. Experiments reveal how changes to the independent variable affect the dependent variable.

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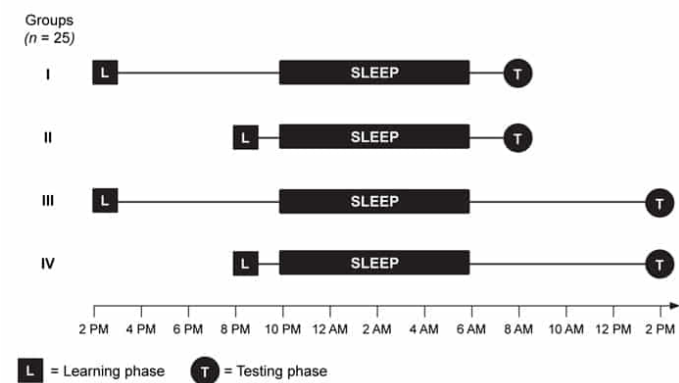


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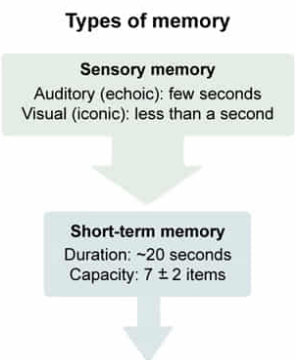
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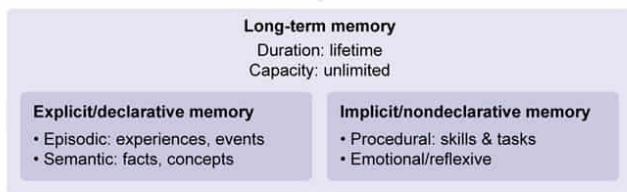
In this study, the vocabulary recall task most directly assessed which of the following types of memory?

- ☐ A. Implicit [3%]
- ☒ B. Semantic [65%]
- ☐ C. Short-term [15%]
- ☐ D. Working [15%]

Correct
65% answered correctly

Explanation:





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Research suggests that humans have 2 major types of **long-term memory**, implicit and explicit. **Explicit/declarative** memory is memory for facts and events that can be **consciously** or intentionally **recalled**, including:

- **Semantic memory**, which includes knowledge about **facts** (eg, humans are mammals)
- **Episodic memory**, which includes **personal experiences** (eg, what you ate for dinner last night)

During the testing phase of this study, students were asked to recall objects they had previously seen (episodic memory) and define vocabulary words they had previously studied (semantic memory). Therefore, the recall tasks in this study were most directly assessing these 2 aspects of explicit/declarative memory.

(Choice A) Implicit/nondeclarative memory is memory for things that cannot be consciously recalled, such as skills, tasks, emotions, and reflexes. Participants were asked to intentionally recall objects and vocabulary definitions, which are examples of explicit/declarative memory; implicit memory was not assessed in this study.

(Choice C) Distinct from long-term memory (which has a large capacity and long duration), short-term memory can hold roughly 5–9 items for about 20 seconds. Short-term memory temporarily stores information, some of which is passed on to long-term memory. This study, which tested recall 12-24 hours after learning, did not assess short-term memory.

(Choice D) Although scholars debate the extent to which working memory and short-term memory are distinct, working memory is the system that manipulates and processes information and focuses attention. Working memory is best assessed with tasks that require attention shifting, information manipulation, or other more complex cognitive processes, which were not included in this study.

Educational objective:

Long-term memory includes implicit/nondeclarative (eg, "how to") and explicit/declarative memory (eg, "what is"). Declarative memory includes semantic memory (eg, facts) and episodic memory (eg, personal experiences).

Time spent:0 sec

Subject:
Behavioral Sciences

QID:400175

System:
Learning, Memory, and Cognition

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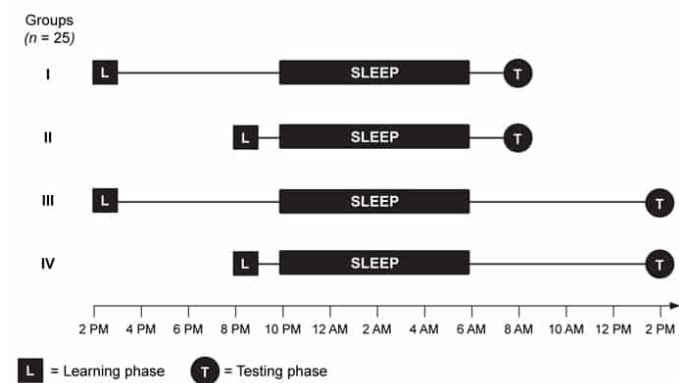


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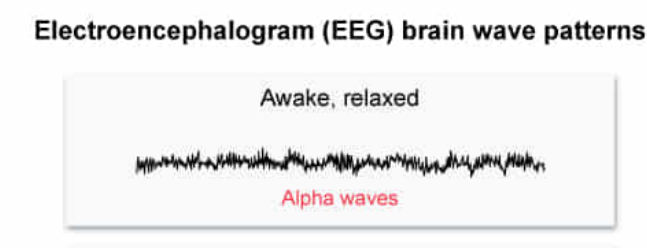
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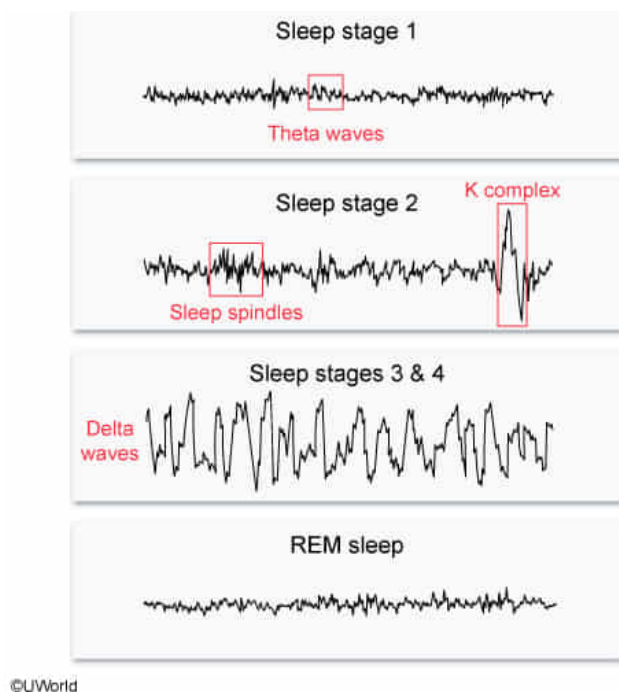
According to the first paragraph, which of the following electroencephalogram patterns should predominate during the sleep stage associated with the consolidation of the type of memory assessed in this study?

- ☐ A. Alpha and beta waves [17%]
- ☐ B. Theta waves [15%]
- ☒ C. Delta waves [57%]
- ☐ D. Sleep spindles and K complexes [9%]

Correct
57% answered correctly

Explanation:





An **electroencephalogram (EEG)** measures brain activity patterns reflective of sleep and waking states. According to the first paragraph, procedural memories are primarily consolidated during REM sleep, whereas *declarative memories* (which were assessed in this study) are primarily consolidated during slow-wave sleep.

The deepest of non-REM sleep, **slow-wave sleep (stages 3 and 4)** is characterized by **delta waves**, which demonstrate the lowest frequency and highest amplitude observed during the sleep cycle.

(Choice A) Alpha and beta waves are high-frequency, low-amplitude waves characteristic of waking states. Beta waves have the highest frequency and are characteristic of awake, alert states. Alpha waves have more regularity and are characteristic of awake, relaxed states.

(Choices B and D) Theta waves are characteristic of stage 1 sleep ("light sleep"). During stage 2 sleep, theta waves still predominate but are interrupted by occasional sleep spindles (bursts in frequency) and K-complexes (increases in wavelength).

Educational objective:

Electroencephalograms measure brain wave patterns that are characteristic of the following states: awake/alert (beta waves), awake/relaxed (alpha waves), stage 1 sleep (theta waves), stage 2 sleep (theta waves interspersed with sleep spindles and K-complexes), stages 3 and 4 slow-wave sleep (delta waves), and REM sleep (high-frequency, low-amplitude

waves that appear similar to beta waves).

Time spent:0 sec

Subject:
Behavioral Sciences

QID:400176

System:
Learning, Memory, and Cognition

A common misconception in medicine is that eating disorders present similarly in males and females. Research suggests that this is untrue: males meeting diagnostic criteria for eating disorders are more likely than females to have another psychiatric illness, demonstrate a later age of onset, and engage in excessive exercise. They are less likely to engage in purging behaviors, be diagnosed by clinicians, or seek treatment. When males do enter treatment, they are generally further along in the course of the illness.

The classification of eating disorders in males according to the *Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition* (DSM-5) has recently been debated. Research suggests that one very common manifestation of eating disorder etiology in males, called muscle dysmorphia (MD), includes an obsession with a larger and more muscular body, which contrasts with the thin, idealized body for females with eating disorders. Characterized as a subset of body dysmorphic disorder, MD is more recently conceptualized as part of a spectrum of eating disorders; however, it is still classified under the obsessive-compulsive and related disorders umbrella in the DSM-5.

One study applied the latest DSM-5 diagnostic criteria for anorexia nervosa (AN), bulimia nervosa (BN), binge eating disorder (BED), and MD to the medical records of male veterans treated in Veterans Administration (VA) hospitals across the United States from 2000 to 2015. The study (Table 1) found a significant increase in psychiatric comorbidity compared to controls (C).

Table 1 Prevalence of Psychiatric Disorders for Eating Disorder and MD Cases and Controls in a National Sample of Male Veterans Treated at VA Hospitals from 2000 to 2015

	AN	BN	BED	MD	C
Comorbid psychiatric diagnosis	(n=20)	(n=14)	(n=22)	(n=36)	(n=92)
Depressive disorders	35%	70%	50%	32%	8%
Anxiety disorders	21%	23%	22%	20%	12%
Substance use & addictive disorders	35%	60%	30%	49%	12%

Alcohol abuse/dependence	30%	40%	24%	22%	9%
Drug abuse/dependence	15%	25%	25%	44%	3%
Schizophrenia spectrum/other psychotic disorders	35%	22%	20%	25%	10%

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The DSM-5 describes AN as characterized by a distorted body image and excessive food restriction leading to severe weight loss. How would a behaviorist explain the behavior of an individual diagnosed with AN?

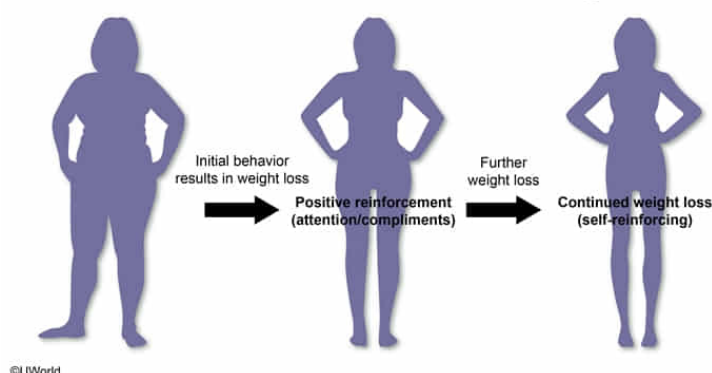
- ☐ A. The eating behavior is evidence of a biochemical imbalance in the dopamine reward system in the brain. [2%]
- ✓ ☒ B. Positive attention from others for initially losing weight motivates the individual to want to lose more weight. [58%]
- ☐ C. The individual is behaving in a way to attain the "thin ideal" portrayed by celebrities in the media. [20%]
- ☐ D. The individual most likely has a close friend or family member with an eating disorder and is imitating that behavior. [18%]

Correct

57% answered correctly

Explanation:

Anorexia nervosa: behaviorist model of weight loss



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Behaviorism is a theory in psychology that focuses on the role the **environment** plays in shaping human behavior. Specifically, behaviorism focuses on how **reinforcement** and **punishment** shape human behavior.

Reinforcement is anything that increases the likelihood of the behavior happening again. For example, positive attention for losing weight would reinforce that behavior and make it more likely the individual will lose more weight. **Punishment** is anything that decreases the likelihood of a behavior happening again. For example, if individuals are ridiculed for a behavior, they are less likely to engage in that behavior again.

Although positive attention for losing weight might initially reinforce the behaviors (eg, food restriction) associated with that weight loss, additional weight loss can become a self-reinforcing cycle in which an individual no longer needs external reinforcement and the weight loss itself is reinforcing.

(Choice A) A behaviorist approach is more likely to be focused on the environmental factors influencing behavior whereas a biological approach is more likely to consider biochemical factors.

(Choice C) A behaviorist might consider the role of idealized body types portrayed in the media and societal standards of beauty but would be more concerned with the direct reinforcements and punishments that shape individual behavior.

(Choice D) A social learning theorist is more likely to suggest that individuals with AN have close friends or family members whose behavior they are imitating. Social learning theory suggests that behavior is shaped through social interaction, imitation, and modeling, and can occur in the *absence* of direct reinforcement.

Educational objective:

Behaviorism focuses on how an individual's behavior is shaped by the environment, particularly through reinforcement and punishment. Reinforcement tends to increase the likelihood of the behavior happening again whereas punishment decreases the likelihood of the behavior happening again.

Time spent:0 sec
Subject:
Behavioral Sciences

QID:400177
System:
Learning, Memory, and Cognition

A common misconception in medicine is that eating disorders present similarly in males and females. Research suggests that this is untrue: males meeting diagnostic criteria for eating disorders are more likely than females to have another psychiatric illness, demonstrate a later age of onset, and engage in excessive exercise. They are less likely to engage in purging behaviors, be diagnosed by clinicians, or seek treatment. When males do enter treatment, they are generally further along in the course of the illness.

The classification of eating disorders in males according to the *Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition* (DSM-5) has recently been debated. Research suggests that one very common manifestation of eating disorder etiology in males, called muscle dysmorphia (MD), includes an obsession with a larger and more muscular body, which contrasts with the thin, idealized body for females with eating disorders. Characterized as a subset of body dysmorphic disorder, MD is more recently conceptualized as part of a spectrum of eating disorders; however, it is still classified under the obsessive-compulsive and related disorders umbrella in the DSM-5.

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Table 1 Prevalence of Psychiatric Disorders for Eating Disorder

and MD Cases and Controls in a National Sample of Male Veterans Treated at VA Hospitals from 2000 to 2015

	AN	BN	BED	MD	C
Comorbid psychiatric diagnosis	(n=20)	(n=14)	(n=22)	(n=36)	(n=92)
Depressive disorders	35%	70%	50%	32%	8%
Anxiety disorders	21%	23%	22%	20%	12%
Substance use & addictive disorders	35%	60%	30%	49%	12%
Alcohol abuse/dependence	30%	40%	24%	22%	9%
Drug abuse/dependence	15%	25%	25%	44%	3%
Schizophrenia spectrum/other psychotic disorders	35%	22%	20%	25%	10%

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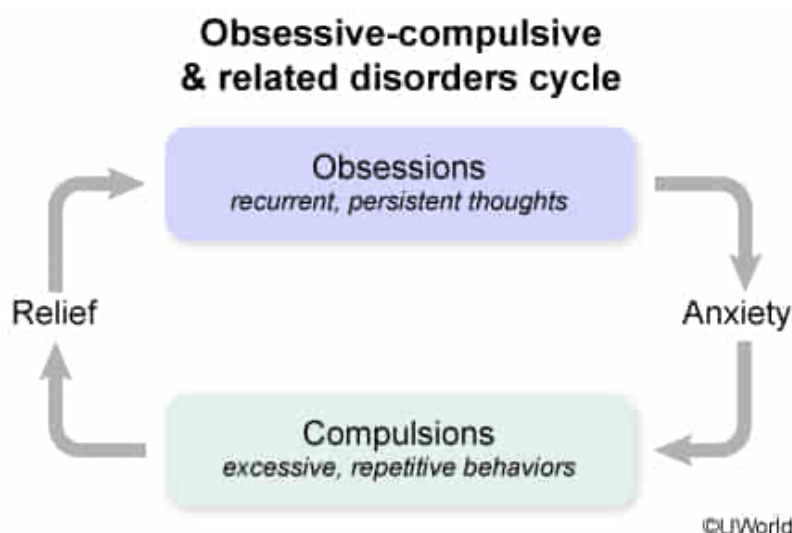
Which is *least* applicable to a diagnosis of MD?

- ☐ A. Persistent, intrusive, and repetitive thoughts or urges [4%]
- ☐ B. Physical behaviors that help reduce anxiety [23%]
- ☐ C. Excessive, repetitive behaviors [3%]
- ✓ ☒ D. Obsession with losing weight [69%]

Correct

69% answered correctly

Explanation:



The passage states that MD is currently classified under the

umbrella of **obsessive-compulsive disorder (OCD)** and **related disorders** under body dysmorphic disorder. OCD and related disorders are characterized by recurrent **obsessions**, which induce anxiety, and **compulsions**, which are used to relieve anxiety.

Obsessions are persistent, intrusive, and repetitive thoughts and urges (**Choice A**). For example, obsessions for an individual with body dysmorphia might include persistent thoughts about being too small. Compulsions are excessive, repetitive behaviors that reduce anxiety (**Choices B and C**). The obsessions and compulsions are significantly time-consuming, interfering with a person's daily routine, social, or work functioning. An individual with OCD or a related disorder may or may not realize that the obsessions and compulsions are excessive and unreasonable.

For example, a compulsion for an individual with body dysmorphia might include weightlifting at the gym for more than 4 hours a day. This behavior (compulsive weightlifting) is meant to relieve the anxiety associated with repetitive thoughts about being too small. MD involves an "obsession with a larger and more muscular body." An obsession with losing weight is the *least applicable* to a diagnosis of MD.

Educational objective:

OCD and related disorders are characterized by obsessions (recurrent, persistent, unwelcome, intrusive, distressing thoughts or urges) and/or compulsions (repetitive behaviors or mental acts meant to neutralize obsessions).

Time spent:0 sec
Subject:
Behavioral Sciences

QID:400178
System:
Learning, Memory, and Cognition

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Table 1 Prevalence of Psychiatric Disorders for Eating Disorder and MD Cases and Controls in a National Sample of Male Veterans Treated at VA Hospitals from 2000 to 2015

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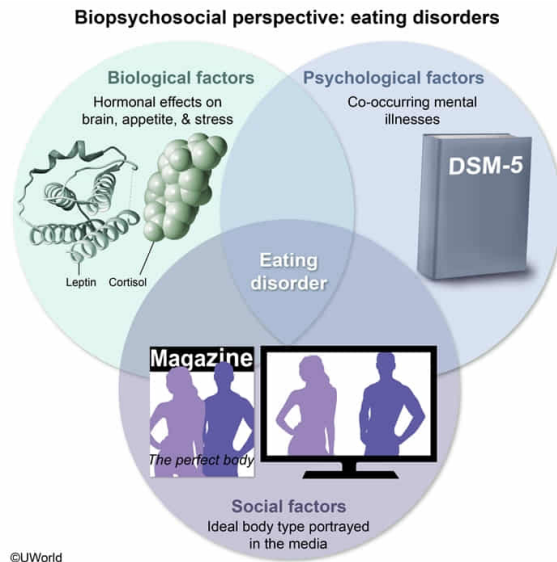
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A proponent of the biopsychosocial approach to mental illness would likely consider which of the following as factors contributing to the development of an eating disorder?

- I. Images seen on television and social media
- II. Variations in hormones such as leptin and cortisol
- III. Co-occurring mental illnesses

- ☐ A. I only [1%]
- ☐ B. I and III only [3%]
- ☐ C. II and III only [7%]
- ☒ D. I, II, and III [88%]

Explanation:



Proponents of the **biopsychosocial** model of mental illness assert that mental illness results from the **interaction** between **biological factors**, **psychological factors**, and **social factors**.

Social factors that might influence the development of mental illness include the social and cultural context in which an individual lives as well as specific factors such as socioeconomic status, family dynamics, peer groups, and major life events. Images portrayed in the media are social factors that could influence the development of an eating disorder.

Biological factors that might influence the development of mental illness include genetic predisposition, hormones, neurotransmitters, anatomy, and physiology. Although potentially overlapping with other categories, certain environmental exposures to pathogens or toxins, physical trauma, and inadequate nutrition can also result in biological changes that make mental illness more likely.

Psychological factors that might influence the development of mental illness include personality characteristics (eg, self-efficacy, negative thought patterns) and co-occurring mental illnesses.

In contrast to the biopsychosocial approach to mental illness, the **biomedical approach** conceptualizes mental illness as directly resulting from chemical imbalances in the brain. The biomedical perspective emphasizes the use of pharmacological therapy to correct these imbalances.

Educational objective:

The biopsychosocial model of mental illness suggests that mental illness is the result of interaction between biological factors, psychological factors, and social factors. This differs from the biomedical model, which suggests that mental illness is caused by chemical imbalances in the brain.

Time spent:0 sec
Subject:
Behavioral Sciences

QID:400179
System:
Learning, Memory, and Cognition

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Which of the following hypothetical findings from a follow-up study evaluating eating disorders in males is most consistent with the data presented in Table 1?

- ☐ A. The proportion of males diagnosed with MD was roughly equivalent to the proportion of females diagnosed with AN. [2%]
- ☐ B. Half of the individuals who met the diagnostic criteria for BED also met the criteria for BN within the past year. [11%]
- ☐ C. Individuals diagnosed with BED were less likely to also meet criteria for depression than individuals diagnosed with AN. [4%]
- ☒ D. Individuals meeting diagnostic criteria for MD were more likely to abuse anabolic steroids than those meeting diagnostic criteria for any other eating disorder. [81%]

Correct

80% answered correctly

Explanation:

Table 1 compares comorbid psychiatric diagnoses (ie, diagnoses that are present simultaneously in one individual) among groups of people diagnosed with eating disorders (AN, BN, and BED) and MD.

According to Table 1, 44% of the 36 cases that met diagnostic criteria for MD also met criteria for drug abuse/dependence, which exceeded the proportion of AN, BN, and BED cases that also met criteria for drug abuse/dependence.

Therefore, the hypothetical finding that individuals with MD were more likely to abuse anabolic steroids than those with any other of the studied disorders is *most consistent* with the data presented in Table 1.

(Choice A) The study described in the passage was conducted using a sample of male veterans; therefore, conclusions about relative proportions of males and females diagnosed with eating disorders are not consistent with Table 1 data.

(Choice B) Table 1 does not provide any information about individuals who may have met diagnostic criteria for more than one eating disorder;

therefore, the finding that those with BED also met the criteria for BN within the past year is not consistent with Table 1 data.

(Choice C) Table 1 suggests that individuals meeting criteria for BED were more likely to also meet criteria for depression (50%) than were those with AN (35%), which *undermines* the finding that individuals diagnosed with BED were less likely to also meet criteria for depression than individuals diagnosed with AN.

Educational objective:

To support findings from another study, data need not report exactly the same findings; rather, results from the first study should align with results from the second.

Time spent:0 sec

QID:400180

Subject:
Behavioral Sciences

System:
Learning, Memory, and Cognition

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A physician suspects that a young woman seen in the clinic displays symptoms of an eating disorder. Which of the following responses best demonstrates the ego defense mechanism of rationalization in the patient?

- ☐ A. The patient states that she writes in her journal or calls a friend whenever she gets the urge to binge and purge. [3%]
- ✓ ☒ B. The patient states that she sometimes skips breakfast and lunch but only when she is planning a big dinner. [73%]
- ☐ C. The patient insists that she does not recall any bingeing or purging episodes, although her mother states that they occur frequently. [13%]
- ☐ D. The patient expresses concern that several of her classmates engage in unhealthy eating patterns and purging behaviors. [9%]

Correct

72% answered correctly

Explanation:

Psychoanalytic theory: major defense mechanisms (incomplete list)

Denial	Refusing or being unable to recognize unacceptable thoughts/behaviors (eg, insisting one is not angry when actually angry)
Projection	Attributing unacceptable thoughts/behaviors to someone or something else (eg, calling the sidewalk "stupid" after tripping)
Rationalization	Making excuses for unacceptable thoughts/behaviors (eg, justifying cheating because "the course is impossible")
Regression	Behaving as if much younger to avoid unacceptable thoughts/behaviors (eg, moving back in with parents to avoid personal responsibilities)
Repression	Blocking unacceptable thoughts/behaviors from consciousness (eg, being unaware of a traumatic past experience)
Displacement	Taking out unacceptable thoughts/behaviors on a safe target (eg, punching a pillow when angry at parents)
Sublimation	Transforming unacceptable thoughts/behaviors into acceptable thoughts/behaviors (eg, taking up boxing as a way to channel one's anger)
Reaction formation	Behaving in a manner opposite unacceptable thoughts/behaviors (eg, expressing love for a person one despises)

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According to psychoanalytic theory, **ego defense mechanisms** are

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an unconscious way to deal with the anxiety caused by unacceptable urges and thoughts. Ego defense mechanisms were first proposed by Sigmund Freud (considered the father of psychoanalytic theory) and later expanded on by other psychoanalytic theorists. There are many defense mechanisms; denial, repression, rationalization, projection, and sublimation are five common mechanisms.

Rationalization occurs when an individual unconsciously generates an acceptable excuse for an unacceptable behavior or thought. For example, skipping meals is not normal or acceptable behavior; the patient using this defense mechanism is rationalizing the behavior by telling herself that she will eat a lot later on.

(Choice A) Sublimation involves turning unacceptable thoughts or behaviors into productive, acceptable ones. An individual who writes in a journal when feeling the urge to binge and purge is using this ego defense mechanism.

(Choice C) Repression involves an inability to consciously recall unacceptable thoughts or behaviors. An individual who is unable to remember engaging in bingeing behaviors is using this ego defense mechanism.

(Choice D) Projection involves the attribution of one's own unacceptable thoughts and behaviors to another person or group. Individuals who focus on the real or imagined unacceptable behaviors of others instead of their own behaviors may be using this ego defense mechanism.

Educational objective:

Psychoanalytic theory suggests that ego defense mechanisms are a method of dealing with unacceptable thoughts and behaviors that cause anxiety. Rationalization is an ego defense mechanism in which an individual generates an acceptable excuse for an unacceptable behavior or thought.

Subject:
Behavioral Sciences

System:
Learning, Memory, and Cognition

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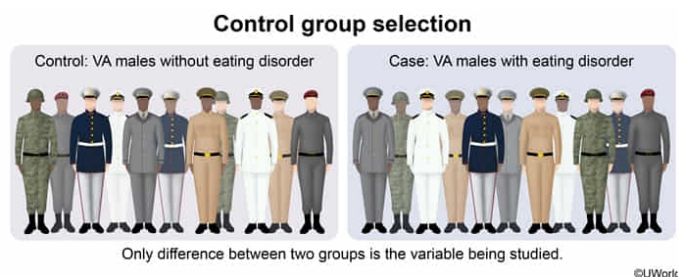
The most appropriate control subjects for the study described in the final paragraph would be:

- ✓ ☒ A. males who have never been diagnosed with an eating disorder. [80%]
- ☐ B. males who have never been treated for an eating disorder. [16%]
- ☐ C. females who have never been diagnosed with an eating disorder. [1%]
- ☐ D. females who have never been treated for an eating disorder.

Correct

80% answered correctly

Explanation:



Control subjects provide a comparison group in experiments. The control subjects should be as similar as possible to the subjects of interest, except for the causal (independent) variable being studied. For example, in clinical trials controls are demographically similar to the treatment subjects (or those under evaluation) but do not receive any active medication or treatment.

This study sought to determine whether a previous diagnosis of an eating disorder would predict other comorbid psychological disorders. The most appropriate control subjects should be similar to other subjects except with regard to the variable being studied. Therefore, the most appropriate control subjects for the study described in the passage are males who have never been diagnosed with an eating disorder.

(Choice B) If the study described in the passage was looking at the rates of psychiatric comorbidity for males who had and had not *received treatment* for their eating disorder, then males who had never been treated

for an eating disorder would be a good comparison group.

(Choices C and D) Control subjects should be as similar to treatment subjects or the observational subjects of interest as possible. Comparing males with eating disorders to females who have never been diagnosed with or treated for an eating disorder would not be an effective comparison because two significant variables (sex and diagnosis/treatment) differ between the groups.

Educational objective:

Control subjects provide a useful comparison to the subjects of interest in a study. Control subjects should be as similar to the treatment subjects as possible, except for the variable of interest.

Time spent:0 sec

QID:400182

Subject:
Behavioral Sciences

System:
Learning, Memory, and Cognition